

FOAM - The Future of Medical Education?

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Introduction

Free Open Access Medical (FOAM) education describes the movement in support of on-line medical education resources that are accessible to all. Exchange of ideas and learning that is “....without fee and covenant” originated within the Hippocratic Oath, but the age of Social Media has brought new capabilities that may be applied to the dissemination of knowledge in non-traditional ways and formats. These include the increasingly popular open-source publishing, blogs, wikis, podcasts and microblogging platforms. FOAM also comprises resources that discuss and debate the outputs of medical research, in addition to those of experiential learnings.

Characteristics	Traditional	FOAM
Quality	↑↓	↑↓
Potential audience size	+	+++++
Accredited	✓	✗
Cost	\$\$\$\$\$\$\$	
Delivery	↑↑↑	↑↑↑

The State of Play

In 2013 there were –

- 270 Critical Care blogs¹
 - Producing 11,841 blog posts
 - Accessed by 3.8 million unique visitors
 - With over 10 million page views
- Eight anaesthesia-specific
 - Two had no content
 - Two authored by non-specialists
 - Two not regularly updated

FOAM Impacts and Actions

Reports of improvements to clinical procedures –

- Delayed sequence intubation with ketamine²

Reports of translation of research findings into clinical practice –

- Targeted temperature management trial³

Enhanced post-publication peer review –

- Correction of peer-reviewed publication⁴

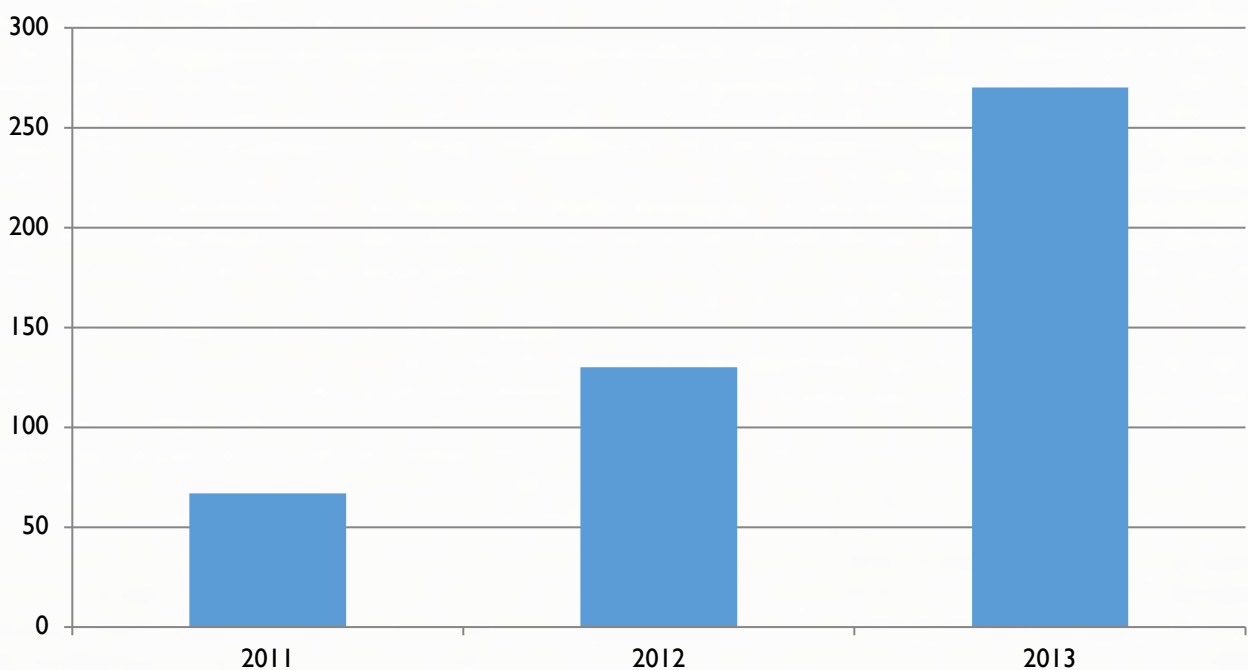
Establishing and fostering networks

- Social Media and Critical Care conference (SMACC)

Missed opportunities for anaesthetic education

- Result of culture of “apprenticeship” training?
- Value of broader exposure to experience of errors

Critical Care blogs by Year



Conclusion

Clearly, open-access on-line resources are already complementing traditional educational programs and are here to stay. Anaesthesia is currently poorly represented in this rich stream of education. To shape medicine of the future, consultants and trainees should be encouraged to be active in FOAM, produce content, and join the conversation.

References

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4. Huang *et al* (2013) NEJM 368: 2255